INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Trastuzumab subcutaneous (SC) · metastatic

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
21 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00272
Version 4

Trastuzumab 600 mg under the skin every 21 days, without a loading dose, while beneficial and tolerated. Any taxane or endocrine partner follows a separate prescription and monitoring plan.

Treatment stages

01 **Current HER2 treatment**
Every 3 weeks

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report breathlessness, swelling, rapid weight gain, palpitations or persistent diarrhoea; tell staff immediately about a reaction during treatment.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

The medicine in this card

Trastuzumab under the skin helps control HER2-positive metastatic breast cancer. It can be used alone or with specified cancer medicines when the full NCCP indication is met.

HER2-TARGETED ANTIBODY

Trastuzumab



Targets HER2 growth signals

Trastuzumab binds to the HER2 protein on cells and helps reduce HER2-driven growth and recruit immune activity. HER2 testing identifies the cancers for which this approach is used.

HER2 on cell surface → Antibody binds → Growth signalling reduced

What to watch for

Keep echo appointments. Report breathing changes, swelling, allergic symptoms or a painful / spreading injection-site reaction.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim

Setting and duration

Trastuzumab 600 mg under the skin every 21 days, without a loading dose, while beneficial and tolerated. Any taxane or endocrine partner follows a separate prescription and monitoring plan.

Medicine identity

This card describes trastuzumab. Pertuzumab, taxanes, endocrine medicines and antibody–drug conjugates require their own treatment instructions.

Trastuzumab formulation evidence

This explains the study setting and its meaning. Ask the team how the evidence applies to you.

Longer follow-up found similar outcomes with the two formulations. This study helps explain the choice of route; it does not predict your own outcome. Its early-cancer outcomes cannot be used as a metastatic survival estimate.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Where this treatment fits

Trastuzumab SC · metastatic

21-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

EVERY 3 WEEKS

Current HER2 treatment

Repeated cycles

Trastuzumab 600 mg under the skin every 21 days, without a loading dose, while beneficial and tolerated. Any taxane or endocrine partner follows a separate prescription and monitoring plan.

Early and metastatic pathways have different aims and endpoints. The team confirms the applicable plan.

BASELINE → TREATMENT → FOLLOW-UP

When your checks happen

Cycle examples assume no delays and count from the first dose of this schedule. If switching route or continuing previous HER2 treatment, preserve the existing echo due date. Tests are scheduled by elapsed time and clinical need; the team owns booking and result review. NCCP remains 12-weekly during treatment, including maintenance; a longer interval needs an explicit individual specialist plan.

Check	Before first dose	Early visits	Week 6 · around C3	Week 12 · around C5	Later / after stopping
Heart pumping test · echo / multigated acquisition (heart pumping) scan (MUGA)	Baseline before starting	Earlier if symptoms or cardiac concern	Not routinely repeated just because week 6 arrives	Repeat at 12 weeks; earlier checks may be needed	Every 12 weeks during treatment; after stopping, the EMA cardiac follow-up schedule is every 6 months until 24 months. Individual cardiac risk may require more.
Blood tests	Baseline full blood count (FBC), renal and liver profile	Additional tests if needed	Repeat before this cycle	Repeat before this cycle	Every 6 weeks

Check	Before first dose	Early visits	Week 6 · around C3	Week 12 · around C5	Later / after stopping
electrocardiogram (ECG) / rhythm	Baseline ECG in EMA cardiac assessment; it does not replace echo.	Symptoms or clinical indication	Clinical plan	Clinical plan	Do not substitute ECG for left ventricular ejection fraction (heart pumping measurement) (LVEF) monitoring
Cancer scans / markers	Document the disease sites and imaging purpose	Assess symptoms at visits	Scan only if due / indicated	Metastatic: often near a response review; exact date agreed	Response scans commonly every 2–4 months; individualise. Markers supplement clinical review and imaging.
dual-energy X-ray absorptiometry (bone density) (DEXA) / bone health	If endocrine / ovarian suppression is planned: assess need	Not a routine trastuzumab test alone	Timing depends on risk / result	Separate from the echo schedule	DEXA and repeat timing depend on endocrine treatment, fracture risk and results.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00272 v4 · official regimen · ESC 2022 cardio-oncology guideline · cardiac surveillance and risk · NICE NG101 · bone health and follow-up · ESMO metastatic breast cancer guideline · response assessment · ASCO · biomarkers in metastatic breast cancer · EMA Herceptin product information · retrieved 25 October 2026

THE WHOLE TREATMENT JOURNEY

Trastuzumab SC · metastatic: treatment and checks

HER2-positive metastatic breast cancer · SC component

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	First cycles	Around 12 weeks	Continuing treatment	Completion / change
Cancer medicine NCCP 00272	Confirm setting, prior doses and formulation	SC trastuzumab every 3 weeks	Review benefit, symptoms and heart function	Continue while beneficial and tolerated	Review benefit, toxicity and goals; continue or change treatment without a fixed one-year end date.
Surgery METASTATIC	Local procedures only if indicated	No automatic curative surgical sequence	Coordinate procedures with oncology	Symptom / site-specific decisions	Local follow-up as indicated
Radiotherapy EARLY · METASTATIC	Assess indication and planned site	Timing depends on the clinical need	Agree any hold / continuation with radiation oncology	radiotherapy (RT) is site / symptom directed. Agree any treatment hold with both teams.	Radiation effects and surveillance follow the local plan
Blood tests NCCP 00272	Baseline FBC, renal, liver	Every 6 weeks	Week 12 is another six-week blood-check point for trastuzumab alone; companion therapy may require more frequent tests.	Every 6 weeks	Follow abnormalities and the next treatment plan
Heart checks NCCP 00272 · SMPC	Baseline echo / MUGA; cardiac assessment	Symptoms can trigger earlier checks	Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles	Repeat every 12 weeks; 4–8 weeks may be needed with impairment	Trastuzumab: six-monthly cardiac checks to 24 months after stopping; see follow-up detail.
Scans / markers EARLY · METASTATIC · MARKERS	Record baseline metastatic sites and scan purpose	Review symptoms at every visit	Response imaging according to the individual disease plan	Usually reassess about every 2–4 months; individualise	Use imaging and clinical assessment together; tumour markers are supplementary.

Care / check	Before treatment	First cycles	Around 12 weeks	Continuing treatment	Completion / change
Endocrine / bone health EARLY · BONE	Check hormone receptor status and endocrine indication	Endocrine timing coordinated with the wider plan	DEXA if indicated by endocrine / fracture risk	May overlap trastuzumab when clinically appropriate	Longer endocrine and bone follow-up can outlast HER2 treatment

What each check is for

1

Blood tests

FBC, renal and liver profile every 6 weeks (around C3, C5, C7 if on time)

Results help determine whether treatment can proceed.

Ask / act: The team confirms booking, results and next dose.

2

Echo / MUGA

Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles

Heart function can change without symptoms.

Ask / act: Keep the booked scan; report symptoms between scans.

3

Cancer imaging and markers

Response imaging commonly every 2–4 months, adapted to sites, disease course and treatment. A new symptom can need earlier imaging. Tumour markers may supplement assessment, never determine a treatment change alone.

Use the result for a specific clinical decision.

Ask / act: Ask which scan is planned and who reviews it.

4

Booking and result ownership

At initiation, each review and handover

A recommended test can be missed without a named owner.

Ask / act: Record booking owner, result reviewer, due window and escalation route.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Your practical treatment plan

STEP 1

Before your visit

Confirm the dose, symptoms, required blood results and echo due window.

STEP 2

Access and administration

SC trastuzumab needs no intravenous (IV) line for this medicine. Another medicine may still need a cannula or port.

STEP 3

Treatment and observation

Injection over 2–5 minutes into alternate thighs. Observation is 30 minutes after the first injection and 15 minutes after subsequent injections, subject to approved local policy.

STEP 4

After the visit

Contact the unit to rearrange a missed dose. SC doses remain at least 3 weeks apart; no loading dose. This medicine does not routinely require a take-home pump.

Injection site

Alternate thighs and avoid tender, bruised, red or hardened skin. Tell the nurse about a persistent or worsening local reaction.

Before treatment and between visits

The team should arrange and review the tests.

Blood checks

FBC, renal and liver profile every 6 weeks (around C3, C5, C7 if on time)

Cardiac checks

Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles. Baseline cardiac assessment includes ECG in EMA trastuzumab information. Further ECGs follow symptoms / cardiac plan; an ECG does not replace an echo.

After stopping

EMA trastuzumab information: cardiac assessment every 6 months after stopping, until 24 months; after anthracyclines in early disease, yearly to 5 years or longer if function falls.

The wider pathway

Coordinate cancer medicines with local treatment and follow-up.



Scans and markers

Response imaging commonly every 2–4 months, adapted to sites, disease course and treatment. A new symptom can need earlier imaging. Tumour markers may supplement assessment, never determine a treatment change alone.

Radiotherapy relationship

Radiotherapy is used for a defined site or symptom when indicated. The oncology and radiation teams agree whether this medicine continues or pauses; do not infer a concurrent schedule from this card.

Bone health

If an aromatase inhibitor or ovarian suppression is planned, assess fracture risk and DEXA need. Repeat timing depends on results and bone treatment; this is not an automatic HER2-therapy scan.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 21: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
SC injection	 **Day 1** Planned dose
Administration reaction	 **Day 1** During / after visit; report promptly
Fatigue / bowel changes	 **Days 1–21** Timing and severity vary
Heart / breathing symptoms	 **Days 1–21** Report on any day

No date entry needed: This is an orientation map; it does not calculate a personal appointment calendar.

Companion medicines: If another cancer medicine is prescribed, its side effects, tests and supportive medicines need its own plan.

Delayed treatment: Blood tests and echoes follow clinical due windows. A postponed dose does not automatically postpone monitoring.

> **Why these bands stay broad:** Treatment days show the planned schedule. Symptom bands are broad possibilities, not measured risk curves or guaranteed safe days. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00272 v4 · official regimen ↗

> **There are no guaranteed "safe days".** New breathlessness, chest pain, collapse, severe allergic symptoms or sudden serious illness need urgent assessment on any day.

Between-treatment notes

Day 1–21: Your reference cycle

Trastuzumab 600 mg under the skin every 21 days, without a loading dose, while beneficial and tolerated. Any taxane or endocrine partner follows a separate prescription and monitoring plan. The team confirms each dose after the relevant checks. Contact them about a missed visit or new symptoms.

Support around the medicines

1 Support for this medicine

Trastuzumab alone has minimal emetogenic risk. Routine chemotherapy antiemetics, steroids and growth factors are not added automatically. Previous reactions require a separate plan.

2 Blood and heart appointments

FBC, renal and liver profile every 6 weeks (around C3, C5, C7 if on time). Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles.

3 Local treatment and endocrine therapy

Surgery and radiotherapy are coordinated by the multidisciplinary team (MDT). When endocrine therapy is indicated, it has its own schedule and bone-health plan.

4 Pregnancy / contraception

Discuss pregnancy, fertility and contraception before treatment. Report pregnancy promptly; do not breastfeed during treatment or for 7 months afterwards. The team confirms the medicine-specific contraception duration.

Questions worth asking

When is my next echo?

Echo / MUGA every 12 weeks; around C5 for uninterrupted 21-day cycles. Existing HER2 exposure and delays affect cycle numbering; the team should give the actual due window.

Does a tumour marker replace a scan?

No. In metastatic disease it may add information to symptoms and imaging. It should not alone determine treatment changes. In symptom-free early disease it is not routine recurrence screening.

Try explaining the plan back: What is the purpose of your treatment? Which heart or blood test is due next, and which team arranges and reviews it?

Trastuzumab

- Heart pumping-function reduction / heart failure
- Allergic or administration-related reactions
- Injection-site pain, redness or swelling
- Diarrhoea, fatigue and rare serious lung effects

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Trastuzumab SC · metastatic

Active medicines: Trastuzumab

Emetogenic risk: Minimal

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- Trastuzumab alone has minimal emetogenic risk. Routine chemotherapy antiemetics, steroids and growth factors are not added automatically. Previous reactions require a separate plan.
- Injection over 2–5 minutes into alternate thighs. Observation is 30 minutes after the first injection and 15 minutes after subsequent injections, subject to approved local policy.
- Contact the unit to rearrange a missed dose. SC doses remain at least 3 weeks apart; no loading dose.
- Companion chemotherapy or endocrine drugs need their own supportive review; this draft covers the named HER2 component only.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Trastuzumab subcutaneous (SC) · metastatic

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Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Breathing

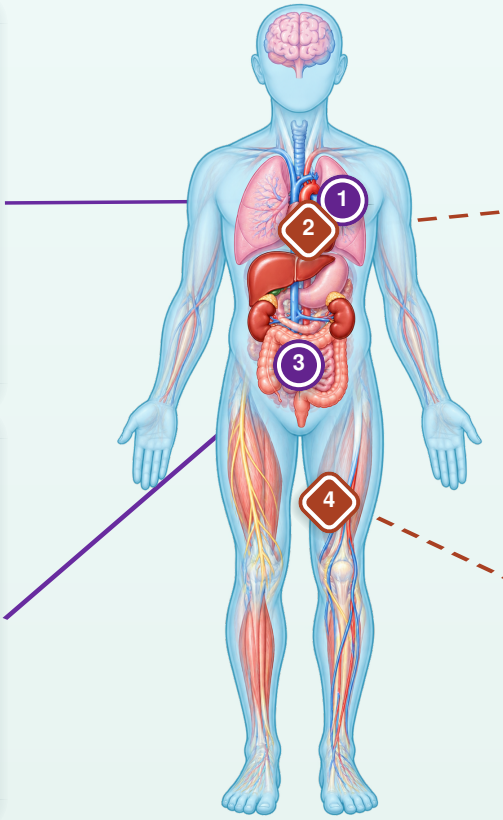
Trastuzumab

New cough or breathlessness: seek prompt assessment.

2 Heart

Trastuzumab

Keep echo checks. Report swelling, chest symptoms, fainting or rapid weight gain.



3 Bowel / fatigue

Trastuzumab

Report persistent diarrhoea, dehydration or fatigue affecting daily activity.

4 Injection site

SC trastuzumab

Report painful, spreading redness or swelling where the injection was given.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

> Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00272 v4 · official regimen ↗**
   https://healthservice.hse.ie/documents/6694/272_v4_Trastuzumab_SC_21days_MBC.pdf
2. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
3. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
4. **ESMO metastatic breast cancer guideline · response assessment**
   https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext
5. **ASCO · biomarkers in metastatic breast cancer**
   https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
6. **EMA Herceptin product information · retrieved 25 October 2026**
   https://www.ema.europa.eu/en/documents/product-information/herceptin-epar-product-information_en.pdf
7. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
8. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
9. **NCCP NCIS antiemetics v8 (2025), Table 1**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
10. **NOGG · section 7, aromatase inhibitors and fracture risk**
    https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
11. **eviQ · implanted port patient information**
    https://www.eviq.org.au/p/507
12. **eviQ · peripherally inserted central catheter (PICC) patient information**
    https://www.eviq.org.au/p/543
13. **HannaH · formulation comparison (2012)**
    https://pubmed.ncbi.nlm.nih.gov/22884505/
14. **HannaH final six-year analysis (2019)**
    https://jamanetwork.com/journals/jamaoncology/fullarticle/2730632